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Project #151: Mobility Matters: Let's Keep Moving Interprofessional Collaboration

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AIM

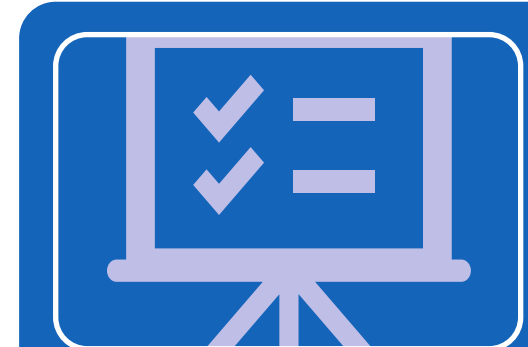
Patients will be mobilized promptly with interprofessional team support and documentation using a common mobility language that is evidence-based to decrease length of stay (LOS) and decrease low value therapy consults.

PLAN: CURRENT STATE

Current Condition: Patients were not being mobilized promptly due to a lack of interprofessional team support and lack of common mobility language amongst care providers leading to increased length of stay (LOS).

- Barriers:** Identified barriers to increasing patients' mobility inpatient
 - Consistent communication with same language gap regarding patients' mobility present
 - Nursing practice gap present by not using a validated Mobility Tool for pre-admission and during admission
 - Physical Therapy (PT) / Occupational Therapy (OT) used other tools: AM-PAC 6-clicks and Johns Hopkins Highest Level of Mobility (both validated tools)
 - Decreased Mobility leads to increased LOS

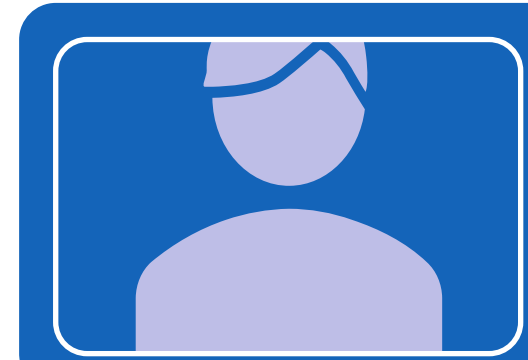
DO: INTERVENTIONS



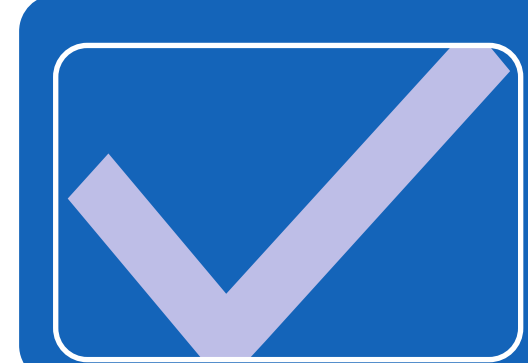
Comprehensive Education Development



Integration of AM-PAC 6-clicks Tool in EMR to Guide Nursing Practice



Interprofessional Collaboration including the Patient and Family Advisory Council (PFAC)



Interprofessional Progressive Rounds include Mobility Status

6 Clicks Basic Mobility	
Turning from your back to your side	
Moving from lying on your back to sitt...	
Sitting down and standing up using ar...	
Moving to and from a bed to a chair (i...	
To walk in hospital room	
Climbing 3-5 steps with a railing	
6 Clicks Basic Mobility Raw Score	

6-Clicks Basic Mobility Scores	
Total (total assist.)	1
A Lot (mod./max. assist.)	2
A Little (min./stand by assist)	3
None (independent)	4

Range	Level of assistance needed
6 - 9	Total functional impairment
10 - 14	Moderate assistance needed
15 - 19	Minimum assistance needed
20 - 22	Supervision needed
23 - 24	Independent

REFERENCES:
Jette, D. U., Silphen, M., Ranganathan, V. K., Passek, S. D., Frost, F. S., & Jette, A. M. (2014). Validity of the AM-PAC "6-Clicks" inpatient daily activity and basic mobility short forms. *Phys Ther*, 94(3), 379-391. doi:10.2522/pt.20130169
Jette DU, Silphen M, Ranganathan VK, Passek S, Frost FS, Jette AM. Interrater reliability of AM-PAC "6-Clicks" Basic Mobility and Daily Activity short forms. *Phys Ther*. 2015;95(5):758-766.
Cape-Lajo C, McLaughlin KH, Ye B, Daley K, Young D, Lavezza A, Friedman M, Hoyer EH. Using Nursing Assessments of Mobility and Activity to Prioritize Patients Most Likely to Need Rehabilitation Services. *Arch Phys Med Rehabil*. 2022;93(10):1402-1408. doi: 10.1016/j.apmr.2023.03.010. Epub 2023 Apr 5. PMID: 37028997.
Hoyer EH, Young DL, Klein LM, et al. Toward a Common Language for Measuring Patient Mobility in the Hospital: Reliability and Construct Validity of Interprofessional Mobility Measures. *Physical Therapy*. 2018;98(2):133-142.
Klein LM, Young D, Fang D, Lavezza A, Hiser S, Daley KN, Hoyer EH. Increasing patient mobility through an individualized goal-centered hospital mobility program: A quasi-experimental quality improvement project. *Nurs Outlook*. 2018 May-Jun;66(5):254-262. doi: 10.1016/j.outlook.2018.02.006. Epub 2018 Feb 27. PMID: 2975382.

ACT: SUSTAIN AND SPREAD

June 2024	Feb 2025	April 2025	
HF Detroit Pilot B1, B4, P6, P4	HF Detroit GPUs	HF Detroit OBS and ICUs	HF System Go Live

Added a Quantitative Method for Interprofessional Communication of the Patient Mobility Status (Pre-Hospital and In-Hospital)

Improved prioritization of Physical Therapy and Occupational Therapy Consults based on scoring

Increased requests to Atlas Mobility Team for both Turning and Out of Bed

RESULTS

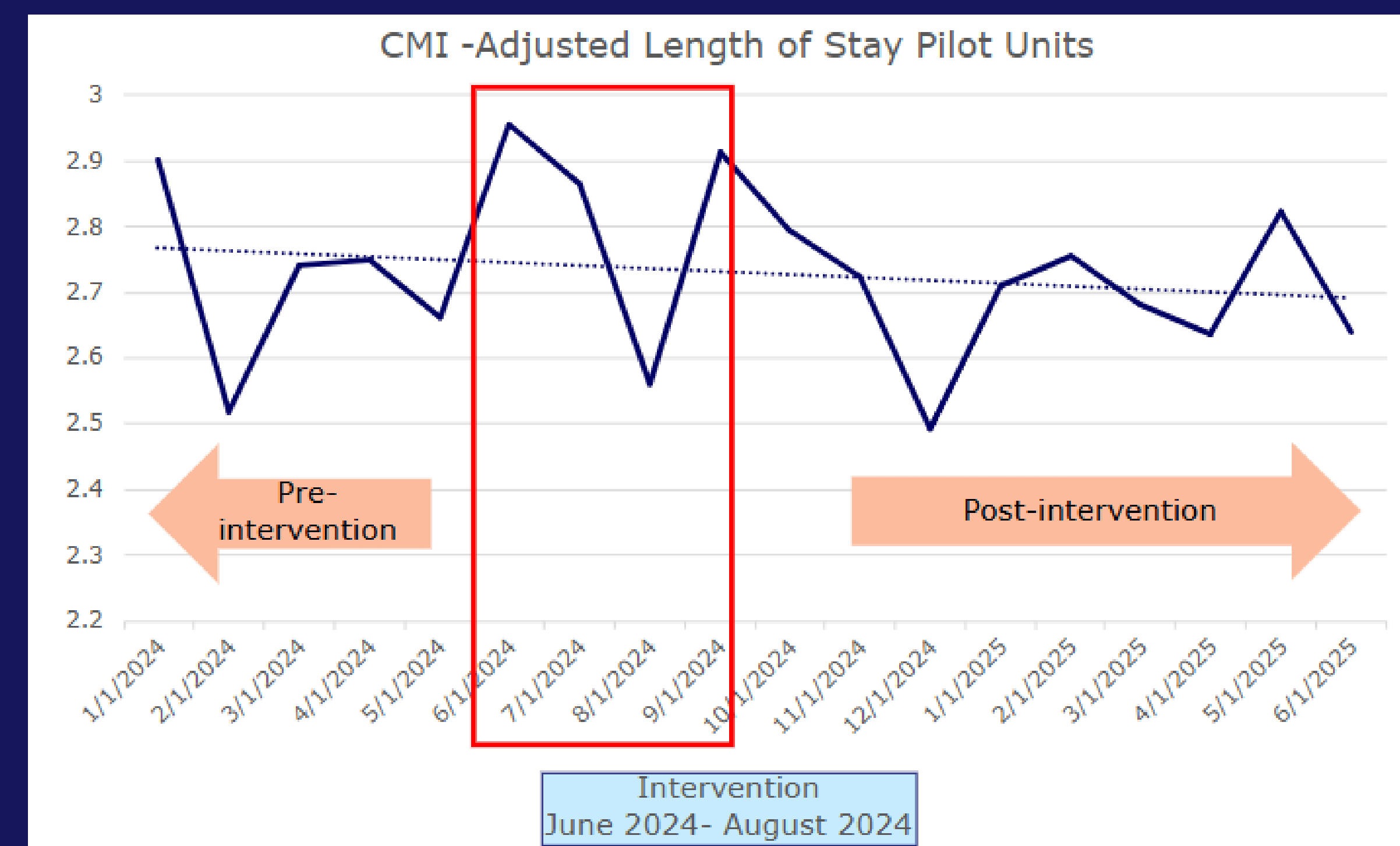


Figure 1: Pilot floors: CMI adjusted LOS reduced from 2.76 to 2.70 days (by 2%)

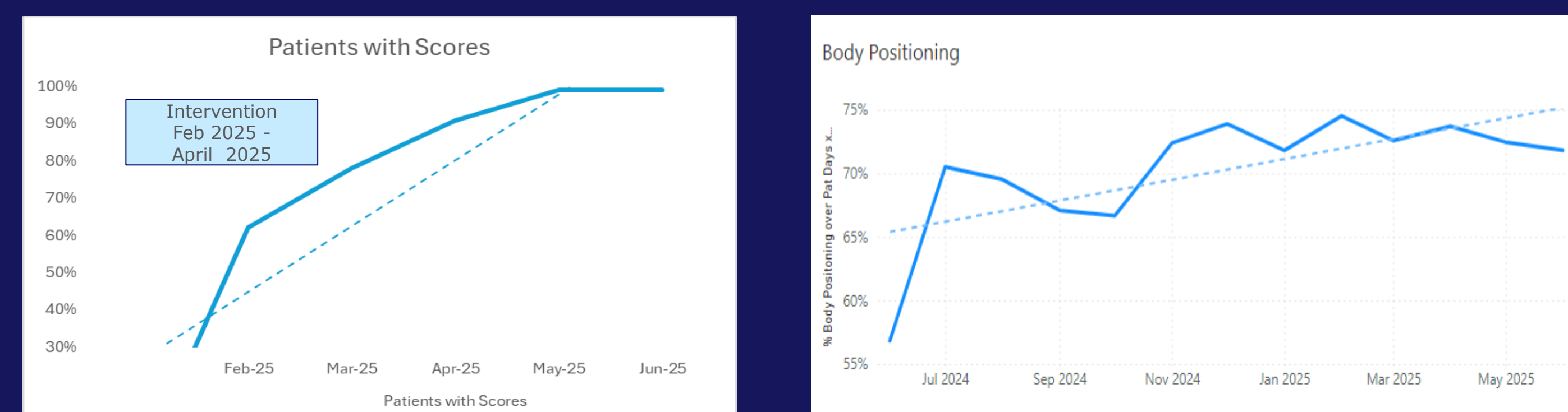


Figure 2: All Floors: RN Scoring compliance increased to >99%

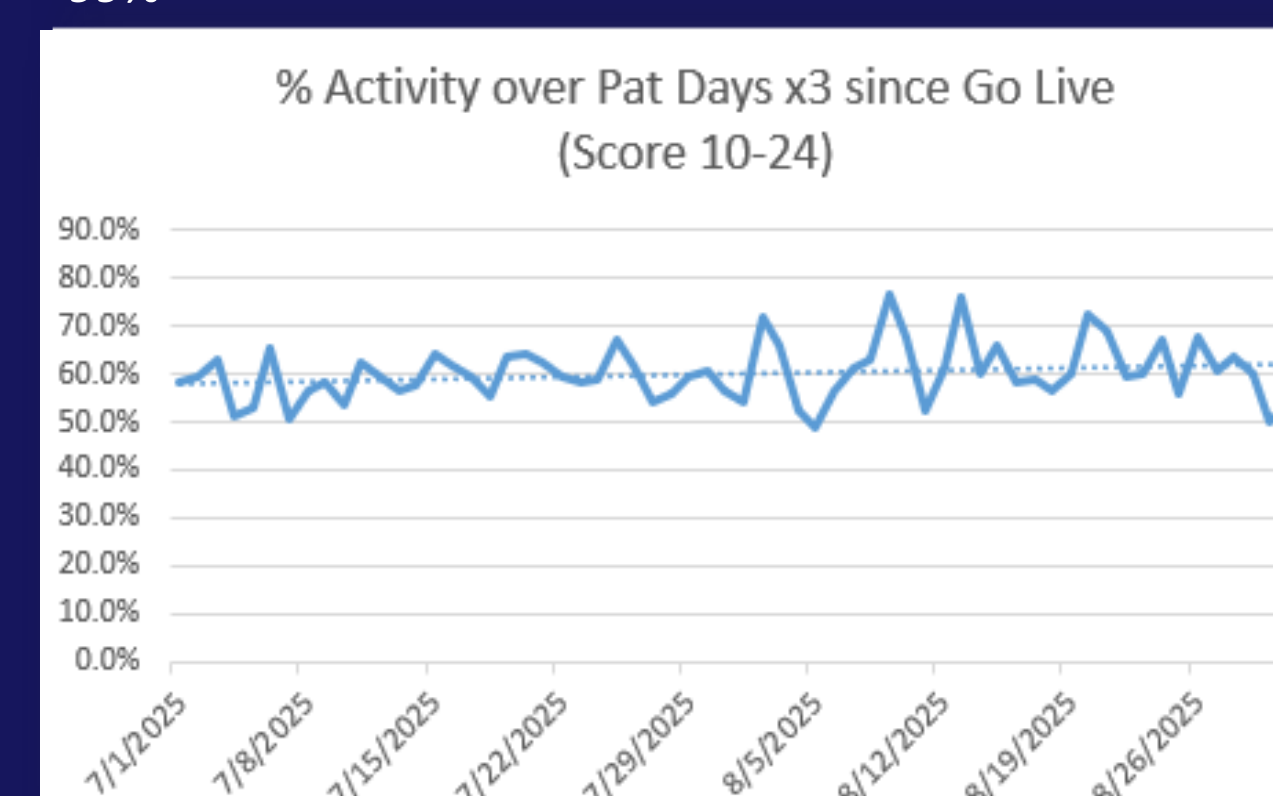


Figure 4: All Floors: For patients with 6 clicks 10-24, Activity out of bed or ambulating 3 times per day

1227
"Low Value Consults"
NOT SEEN (June 2024-August 2025)

Figure 5. Patients scored 23-24 by RN with PT Consults that were NOT seen

Data Sources: Power BI Clinical-Quality Nursing Mobility Matters; Data as of 9/2/25, 11:29 AM. EncounterHsp (is HFH)

ADDITIONAL VISUALS

Welcome to P6 Surgical Specialty & Progressive Care

Room: **P611**
Phone: 313-916-1199

Today's Date: _____
Mon / Tue/ Wed/ Thurs / Fri / Sat / Sun

RN: _____
NA: _____
Provider: _____
Expected Discharge Date: _____

Patient/ Family Notes:

CHG/Nozin

Diet:

Pain:

Daily Goal:

Fall Prevention

STRICT BEDREST until: _____
Weight Bearing Precautions: _____

HELP NEEDED

Complete Goals

Score: 6 - 7

☐ Turn every 2 hours with help.
☐ Range of motion exercises with help (3 times).

Score: 8 - 9

☐ Turn every 2 hours with help.
☐ Sit on side of bed 3 times a day

Score: 10 - 15

☐ Up to a chair 3 times a day

Score: 16 - 17

☐ Stand at side of bed for 1 min or more (3 times)
☐ Up to a chair (3 times)

Score: 18 - 21

☐ Walk to the bathroom or about 10 feet (3 times).
☐ Up to a chair (3 times)

Score: 22 - 23

☐ Walk a hallway or 25 feet (3 times)
☐ Up to a chair (3 times)

Score: 24

☐ Walk unit (3 times)
☐ Up to a chair (3 times)

Figure 6: Signage in each patient room

KEYS TO SUCCESS/ LESSONS LEARNED

- Interprofessional Teams from Physical and Occupational Therapy, Nursing Administration, Nursing, Case Management and Providers.
- Educational Materials are on OneHenry.
- Comprehensive Education Tool Kit was developed which includes:
 - Training Slides
 - Case Studies
 - Competency Check Off Sheet
 - Epic Training Documents
 - Quick Reference sheet
 - Unit RN Binder resources
 - FAQ document for RNs/Nurse Assistants (NA)
- Patient Education materials were created with feedback from the Patient and Family Advisory Council (PFAC).

Mobility Matters Resources

<https://onehenry.hfhs.org/>

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